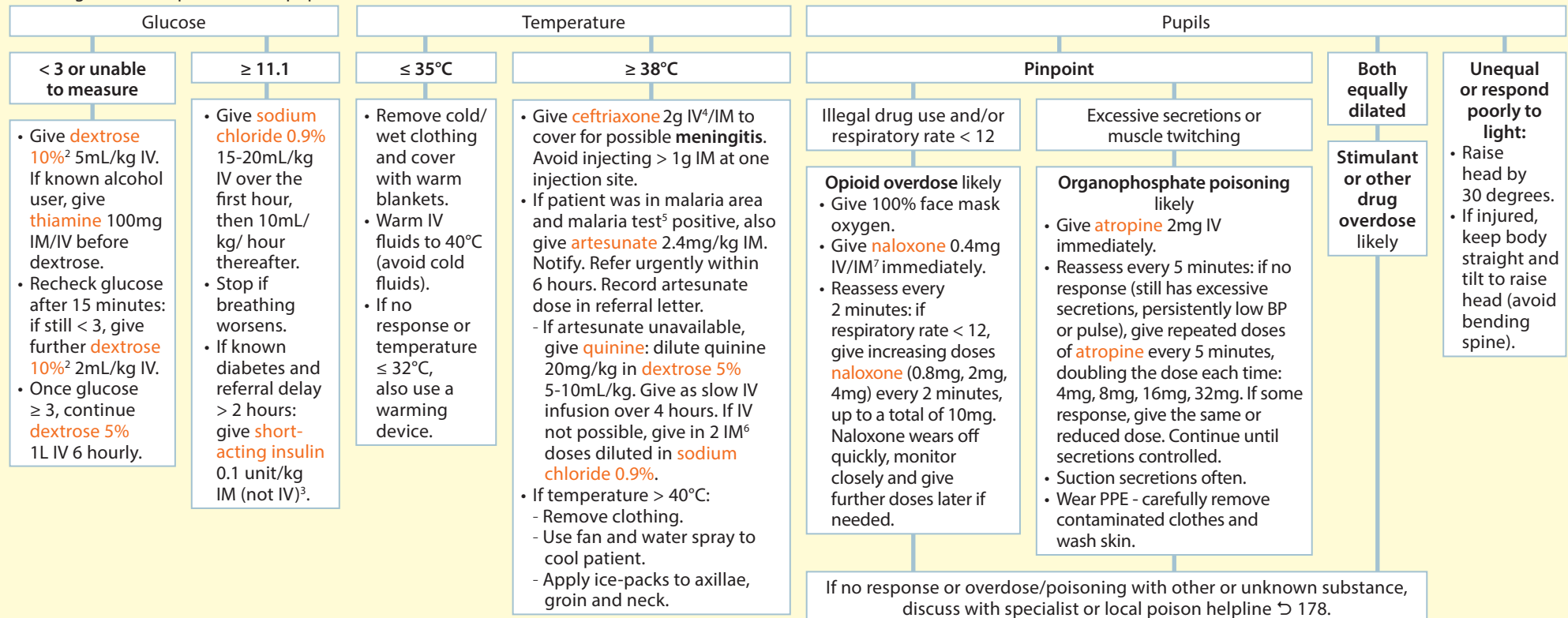


DECREASED CONSCIOUSNESS

Give urgent attention to the patient with decreased consciousness:

- First assess and manage airway, breathing, circulation and level of consciousness $\hookrightarrow$ 14.
- Identify all injuries and look for cause: undress patient and assess front and back. If injured, use log-roll to turn. Then cover and keep warm.
- If fits, injuries or burns, also manage on symptom pages.
- If sudden decreased consciousness and any of: generalised itch/rash, face/tongue swelling, wheeze, difficulty breathing, abdominal pain, vomiting or exposure to possible allergen¹, check for anaphylaxis $\hookrightarrow$ 20.
- Check glucose, temperature and pupils:



- Refer urgently.
- While awaiting transport:
 - Check BP, pulse, respiratory rate, oxygen saturation and GCS every 15 minutes. Insert urinary catheter.
 - If BP < 90/60, pulse > 100 or < 50, respiratory rate > 20 or < 9, oxygen saturation < 94% or drop in GCS, reassess and manage airway, breathing, circulation and level of consciousness $\hookrightarrow$ 14.

¹Common allergens include medication, food or insect bite/sting within the past few hours. ²If dextrose 10% unavailable: mix 1 part **dextrose 50%** to 4 parts water for injection to make dextrose 10% solution. ³Avoid IV insulin as it may cause low potassium and heart dysrhythmia. Avoid using an insulin needle to give IM insulin. ⁴Do not mix Ringer's lactate and IV ceftriaxone. Flush IV line with **sodium chloride 0.9%** before and after IV ceftriaxone. ⁵Test for malaria with rapid diagnostic test if available, and parasite slide microscopy. ⁶To give IM quinine: first calculate volume of **sodium chloride 0.9%** in mL: weight x 20 $\div$ 100. Then add this volume of sodium chloride 0.9% to **quinine** 20mg/kg and inject half the volume into each thigh. ⁷Give naloxone IM only if IV not possible.